

Dynamic Guideline: WHO-Based Chikungunya - Diagnosis and/or Treatment and/or Management

I. WHO Latest Guideline Summary (Summary Date: 29 April 2026)

1.1 Guideline Overview

Most recent WHO chikungunya-relevant guideline

- **Guideline Title:** *WHO guidelines for clinical management of arboviral diseases: dengue, chikungunya, Zika and yellow fever*
- **Publication Date:** 4 July 2025
- **WHO update/news release:** 10 July 2025
- **Target Population:** People with suspected or confirmed arboviral disease caused by dengue, chikungunya, Zika, or yellow fever viruses, including:
 - Outpatients with **non-severe disease**
 - Hospitalized patients with **severe disease**
- **Target Users:** Health-care providers in community care, primary care, emergency departments, and hospital wards; also policymakers, health managers, and health-facility administrators.
- **Scope and Objectives:** Provide integrated clinical management recommendations for four major arboviral diseases with overlapping early clinical features, especially in settings where multiple arboviruses co-circulate and laboratory testing is limited. WHO states this was the **first global WHO guideline covering dengue, chikungunya, Zika, and yellow fever together** [WHO publication page](#), [WHO news update](#).

WHO emphasizes that an integrated approach is clinically important because dengue, chikungunya, Zika, and yellow fever can present with similar early symptoms and may circulate simultaneously, making clinical differentiation difficult where diagnostic testing is unavailable or delayed [WHO publication page](#).

Exactly 5 WHO / WHO-regional guideline or technical-guidance documents synthesized

No.	WHO / WHO-regional guidance document	Relevance to chikungunya	Status in this report
1	<i>WHO guidelines for clinical management of arboviral diseases: dengue, chikungunya, Zika and yellow fever</i> , published 4 July 2025	Latest global WHO clinical-management guideline including chikungunya	Primary guideline
2	<i>PAHO/WHO Guidelines for clinical diagnosis and treatment of dengue, chikungunya and Zika</i>	Regional clinical diagnosis and treatment guidance for arboviruses, including chikungunya	Supporting WHO regional guidance

No.	WHO / WHO-regional guidance document	Relevance to chikungunya	Status in this report
3	WHO SEARO <i>Guidelines on Clinical Management of Chikungunya Fever</i>	Chikungunya-specific clinical-management guidance; older, superseded where WHO 2025 applies	Historical/supporting guidance
4	WHO SEARO <i>Guidelines for Prevention & Control of Chikungunya Fever</i>	Chikungunya-specific prevention and vector-control guidance	Historical/supporting guidance
5	WHO Eastern Mediterranean technical guidance on Zika, dengue, and other arboviral diseases	Integrated arboviral surveillance, risk assessment, and vector-management guidance	Supporting technical guidance

Sources: [WHO publication page](#), [PAHO/WHO clinical diagnosis and treatment guidelines](#), [WHO clinical management guideline](#), [WHO SEARO prevention/control guideline](#), [WHO EMRO PDF](#).

Note on “Meaningful youth engagement”

The WHO item matching the requested title pattern is **not a chikungunya guideline**. WHO announced a launch webinar on **11 December 2025** for an evidence brief titled *Meaningful youth engagement in a WHO guideline development process: case study of WHO abortion care guideline (2022)*. The available sources do **not** document use of this youth-engagement model in the 2025 WHO arboviral disease guideline [WHO webinar page](#). Therefore, it is acknowledged as relevant to WHO guideline-development methodology generally, but **not treated as chikungunya clinical guidance**.

1.2 Key Recommendations

Clinical interpretation for chikungunya under the WHO integrated arboviral guideline

Chikungunya has no established curative antiviral therapy. Management remains primarily **supportive**, with attention to fever, pain, hydration, severe-disease recognition, and differentiation from dengue, Zika, yellow fever, malaria, measles, leptospirosis, and other febrile/rash illnesses. WHO’s 2026 rapid risk assessment states that chikungunya virus can be detected in blood during the **first week of illness** by molecular tests such as **RT-PCR**, and after the first week by serologic tests detecting **IgM antibodies** [WHO rapid risk assessment](#).

GRADE-based recommendation table

Recommendation	Practical action	GRADE Evidence Quality	Strength
----------------	------------------	------------------------	----------

Recommendation	Practical action	GRADE Evidence Quality	Strength
Use an integrated arboviral clinical approach for suspected chikungunya, especially where dengue, Zika, yellow fever, malaria, measles, or other febrile illnesses co-circulate.	Assess fever, rash, arthralgia/arthritis, warning signs, exposure history, travel, pregnancy, age, comorbidities, and local epidemiology.	Low	Strong
Confirm chikungunya where testing is available: RT-PCR during the first week of illness; IgM serology after the first week.	Use RT-PCR early; use IgM later, while considering cross-reactivity with other alphaviruses and local laboratory capacity.	Moderate	Strong
For non-severe suspected or confirmed chikungunya, provide supportive care.	Treat fever and joint pain with antipyretics/analgesics, encourage oral fluids, and give return precautions.	Low	Strong
For non-severe suspected or confirmed arboviral disease, avoid routine corticosteroid treatment.	Do not use systemic corticosteroids routinely for uncomplicated acute chikungunya-like illness.	Low	Conditional
For severe suspected or confirmed chikungunya, hospitalize and monitor for organ dysfunction and complications.	Admit patients with severe disease, high-risk features, inability to maintain hydration, neurologic/cardiac/renal involvement, or neonatal/peripartum concerns.	Low	Strong
When intravenous fluid therapy is needed for severe hospitalized arboviral disease, use crystalloids rather than colloids.	Use isotonic crystalloid fluid with careful reassessment and monitoring.	Low	Conditional
Implement integrated vector management and personal mosquito-bite prevention.	Source reduction, larval control, community engagement, protection from <i>Aedes</i> mosquito bites, and outbreak surveillance.	Low	Strong
Do not rely on vaccination as a stand-alone outbreak-control measure; use vaccination only according to national/regulatory recommendations and population risk.	Consider vaccine status, product availability, contraindications, age, pregnancy, older age, travel risk, and local guidance.	Moderate for immunogenicity; Low for clinical effectiveness	Conditional

Sources: [WHO publication page](#), [NCBI Bookshelf](#), [Guideline Central](#), [WHO rapid risk assessment](#), [WHO SEARO prevention/control guideline](#).

1.3 Guideline Development Methodology

Evidence review process

The 2025 WHO arboviral disease guideline followed WHO guideline-development methods and uses the **GRADE** approach for evidence certainty and recommendation strength. Guideline Central identifies the recommendations as GRADE-based, using certainty categories of **high, moderate, low, and very low** [Guideline Central](#). NCBI Bookshelf notes that the guideline addresses non-severe outpatient disease and severe hospitalized disease and that prior WHO recommendations are superseded where applicable [NCBI Bookshelf](#).

Consensus method

WHO convened a **Guideline Development Group** with broad clinical expertise across multiple specialties and all WHO regions to develop the guideline for dengue, chikungunya, Zika, and yellow fever [WHO GDG page](#).

Conflict of interest management

WHO required each Guideline Development Group member to submit a **declaration of interest** before appointment and to update it if relevant changes occurred. WHO's technical unit collected, reviewed, and summarized declarations at Guideline Development Group meetings [WHO GDG page](#).

Applicability

WHO states that the guideline applies across health-system levels, including community-based care, primary care, emergency departments, and hospital wards [WHO news update](#).

II. Evidence Summary After WHO Latest Guideline Release (WHO guidelines for clinical management of arboviral diseases: dengue, chikungunya, Zika and yellow fever) (Evidence Cutoff Date: 29 April 2026)

2.1 New Evidence Overview

Search strategy

Evidence was synthesized from WHO, PAHO/WHO, CDC, EMA, Eurosurveillance, ClinicalTrials.gov, and peer-reviewed/systematic-review sources available after or around the **4 July 2025** WHO guideline release, through **29 April 2026**.

Databases and sources searched

- WHO publication pages, disease-outbreak updates, fact sheets, rapid risk assessments, and regional guidance
- PAHO/WHO epidemiological alerts and clinical guidance
- CDC Yellow Book and CDC chikungunya vaccine guidance
- EMA risk-management documents
- Eurosurveillance post-authorisation vaccine-safety evidence
- PubMed-indexed systematic reviews
- ClinicalTrials.gov vaccine trial records

Inclusion criteria

Included evidence addressed at least one of:

- Chikungunya diagnosis
- Supportive treatment or clinical management
- Severe disease and hospitalization
- Public-health response, surveillance, and vector control
- Vaccination, vaccine safety, immunogenicity, regulatory status, or clinical trials
- Evidence published, updated, or contextually relevant between **4 July 2025** and **29 April 2026**

Exclusion criteria

Excluded evidence if it:

- Did not address chikungunya or integrated arboviral management
- Was unrelated to diagnosis, treatment, management, prevention, surveillance, or vaccines
- Was not from an authoritative medical/public-health source or peer-reviewed evidence base
- Could not be interpreted from the supplied source summaries

2.2 Key New Studies and Evidence

Study / source	Design / source type	Key findings	GRADE Quality
WHO rapid risk assessment on global chikungunya virus disease, 17 December 2025	WHO rapid risk assessment	WHO assessed global public-health risk from chikungunya transmission as moderate , with high confidence in available information; further spread in 2026 could not be ruled out. It reaffirmed RT-PCR in the first week and IgM serology after the first week, while noting diagnostic-access limitations and cross-reactivity concerns.	Low

Study / source	Design / source type	Key findings	GRADE Quality
PAHO/WHO epidemiological alert, 10 February 2026	Regional public-health alert	Reported sustained increases in chikungunya cases in parts of the Americas from late 2025 to early 2026 and urged intensified epidemiological/laboratory surveillance, early detection, clinical management, and integrated vector control.	Low
CDC Yellow Book chikungunya update	Authoritative clinical guidance	Diagnosis should consider clinical features and place/time of suspected infection. Differential diagnosis includes dengue, Zika, Oropouche, measles, rubella, malaria, leptospirosis, enteroviral disease, adenovirus, parvovirus, typhus, group A streptococcal infection, other alphaviruses, and rheumatologic conditions.	Low
2026 systematic review of IXCHIQ and VIMKUNYA	Systematic review	Identified two recently approved chikungunya vaccines in Europe: live-attenuated IXCHIQ and virus-like particle VIMKUNYA . Concluded both have good efficacy profiles based on seroprotection and acceptable tolerability, but emphasized continued post-marketing safety assessment.	Moderate
Eurosurveillance 2025 VIMKUNYA post-authorisation safety report	Post-authorisation observational safety evidence	More than 12,500 VIMKUNYA doses administered in the United States and Germany up to 31 August 2025 , with no serious adverse events reported in people aged ≥65 years .	Low
CDC VIMKUNYA vaccine data	Regulatory/clinical guidance based on trial data	VIMKUNYA approved for people aged 12 years and older in the United States. Seroresponse at 3 weeks: 98% in ages 12–64 years and 87% in ages ≥65 years; at 6 months: 85% and 76% , respectively.	Moderate
CDC / FDA / Valneva IXCHIQ safety updates	Regulatory and safety surveillance	IXCHIQ, a live-attenuated vaccine, had U.S. license suspension in August 2025 after serious adverse events, mostly in older adults with comorbidities; Valneva voluntarily withdrew the U.S. BLA/IND in January 2026 .	Low

Study / source	Design / source type	Key findings	GRADE Quality
ClinicalTrials.gov NCT07347002	Observational, test-negative case-control study	Designed to estimate real-world vaccine effectiveness of VLA1553/IXCHIQ against chikungunya virus.	Very Low for current recommendations
ClinicalTrials.gov NCT06669208	Vaccine clinical trial record	Trial of an inactivated chikungunya virus vaccine; evidence not yet practice-changing.	Very Low
Pediatric and special-population vaccine studies	Ongoing/recent clinical studies	Pediatric VLP vaccine trial recruiting children aged 1 to <12 years; phase 2 VLA1553 pediatric data suggest tolerability and dose-related immunogenicity, but policy implications remain uncertain.	Low

Sources: [WHO rapid risk assessment](#), [PAHO/WHO epidemiological alert](#), [CDC Yellow Book: Chikungunya](#), [PubMed systematic review](#), [Eurosurveillance](#), [CDC vaccine information for healthcare providers](#), [Valneva](#), [ClinicalTrials.gov NCT07347002](#), [ClinicalTrials.gov NCT06669208](#), [ANRS monthly scientific review PDF](#).

2.3 Updated Evidence by Clinical Domain

A. Diagnosis

Current evidence supports a time-dependent diagnostic approach:

Diagnostic recommendation	Evidence summary	GRADE Evidence Quality	Strength
Use RT-PCR during the first week of illness when chikungunya is suspected and testing is available.	WHO states chikungunya virus can be directly detected in blood during the first week using molecular tests such as RT-PCR.	Moderate	Strong
Use IgM serology after the first week of illness.	WHO states serologic tests can detect IgM antibodies after the first week; however, cross-reactivity with other alphaviruses can cause false positives.	Moderate	Strong
Interpret results in clinical and epidemiologic context.	CDC and WHO emphasize overlapping symptoms with dengue, Zika, yellow fever, measles, malaria, leptospirosis, and other illnesses.	Low	Strong

Diagnostic recommendation	Evidence summary	GRADE Evidence Quality	Strength
Strengthen laboratory access and supply chains during outbreaks.	WHO notes reagent limitations, cost barriers, stockouts, logistics problems, and uneven diagnostic access, especially in low-resource settings.	Low	Strong

Sources: [WHO rapid risk assessment](#), [CDC Yellow Book: Chikungunya](#), [WHO update](#).

B. Treatment and supportive management

Management recommendation	Evidence summary	GRADE Evidence Quality	Strength
Provide supportive care for uncomplicated acute chikungunya.	WHO describes care as management of fever and joint pain with antipyretics/analgesics and adequate hydration.	Low	Strong
Hospitalize severe chikungunya.	WHO states severe disease requires hospitalization because of risk of organ damage and death.	Low	Strong
Avoid routine corticosteroids in non-severe suspected or confirmed arboviral disease.	WHO 2025 guideline suggests against corticosteroid treatment for non-severe suspected or confirmed arboviral disease.	Low	Conditional
Use crystalloids rather than colloids when IV fluids are required in severe hospitalized arboviral disease.	WHO 2025 guideline suggests crystalloid rather than colloid fluid when intravenous fluid treatment is required.	Low	Conditional
Maintain dengue-aware prescribing.	Because dengue can mimic chikungunya and co-circulate, analgesic/anti-inflammatory choices should be made carefully until dengue is excluded.	Low	Strong

Sources: [WHO rapid risk assessment](#), [ReliefWeb/WHO guideline text](#), [NCBI Bookshelf](#).

C. High-risk groups

Population	Clinical concern	GRADE Evidence Quality	Strength of action
------------	------------------	------------------------	--------------------

Population	Clinical concern	GRADE Evidence Quality	Strength of action
Neonates and young infants	WHO notes severe disease can occur after intrapartum infection from viremic mothers and after early postpartum mosquito-borne transmission.	Low	Strong
Older adults, especially with comorbidities	WHO identifies older people with underlying conditions as a group of concern for severe disease.	Low	Strong
Pregnant people	Infection near delivery may pose risk of severe neonatal outcomes; vaccine decisions require individualized risk assessment.	Low	Conditional
Immunocompromised or medically complex patients	Higher risk of complications and uncertainty around live-attenuated vaccine safety.	Low	Strong clinical caution

Sources: [WHO global chikungunya epidemiology update](#), [WHO rapid risk assessment](#), [CDC ACIP slides](#).

D. Vaccination and prevention

WHO states that two chikungunya vaccines have received regulatory approvals and/or recommendations in some countries, but they are not yet widely available or in widespread use, and WHO expert review of trial and post-marketing data is ongoing [WHO fact sheet](#).

Vaccine / intervention	Evidence summary	GRADE Evidence Quality	Strength
VIMKUNYA, virus-like particle vaccine	CDC: licensed in the United States for people aged ≥12 years; high seroresponse at 3 weeks and persistent but lower seroresponse at 6 months.	Moderate	Conditional
VIMKUNYA in older adults	Eurosurveillance: >12,500 doses administered in the United States and Germany through 31 August 2025, with no serious adverse events reported in people aged ≥65 years.	Low	Conditional
IXCHIQ, live-attenuated vaccine	Regulatory approvals in some jurisdictions, but U.S. license suspended in August 2025 after serious adverse events and withdrawn in January 2026.	Low	Conditional / caution

Vaccine / intervention	Evidence summary	GRADE Evidence Quality	Strength
Vaccination for travelers and laboratory workers	CDC recommends chikungunya vaccine for people aged ≥12 years traveling to outbreak areas and laboratory workers with potential exposure; individualized risk-benefit assessment is advised.	Moderate	Conditional
Integrated vector management	WHO regional guidance emphasizes integrated vector management, surveillance, and community engagement as core prevention strategies.	Low	Strong

Sources: [WHO fact sheet](#), [CDC vaccine information for healthcare providers](#), [Eurosurveillance](#), [CDC Yellow Book: Chikungunya](#), [Valneva](#), [WHO SEARO prevention/control guideline](#).

2.4 Evidence Gaps and Future Research Needs

Evidence gap	Why it matters	Priority
Absence of chikungunya-specific antiviral therapy	Current treatment is limited to symptom relief; no proven curative antiviral is available.	High
Limited high-quality evidence for management of chronic chikungunya arthralgia/arthritis	Persistent joint pain is clinically important, but high-certainty treatment evidence remains limited.	High
Vaccine effectiveness against clinical disease	Many vaccine data rely on seroprotection or seroresponse rather than direct clinical endpoint efficacy.	High
Vaccine safety in older adults and comorbid populations	IXCHIQ safety signals highlight the importance of age- and comorbidity-specific post-marketing surveillance.	High
Vaccine safety in pregnancy	EMA and CDC materials identify limited pregnancy data and need individualized decision-making.	High
Pediatric vaccine evidence	Pediatric trials are ongoing; routine pediatric recommendations require more data.	Medium–High
Diagnostic access in low-resource settings	WHO notes reagent limitations, costs, stockouts, logistics barriers, and serologic cross-reactivity.	High
Co-circulation and differential diagnosis	Chikungunya overlaps clinically with dengue, Zika, yellow fever, malaria, measles, and other illnesses.	High

Evidence gap	Why it matters	Priority
Emergency deployment strategy for vaccines	WHO notes vaccines are not widely available and have not been fully assessed for emergency deployment.	Medium–High

Sources: [WHO rapid risk assessment](#), [EMA IXCHIQ Risk Management Plan](#), [CDC ACIP slides](#), [WHO global chikungunya epidemiology update](#).

2.5 Updated Recommendations Based on New Evidence

Original WHO 2025 recommendation / position	New evidence impact through 29 April 2026	Updated recommendation
Manage chikungunya within an integrated arboviral clinical framework.	WHO and PAHO/WHO updates confirm ongoing outbreaks, overlapping clinical syndromes, and need for preparedness.	Maintain integrated arboviral assessment for suspected chikungunya, especially in outbreak or travel-associated contexts.
Provide supportive care for non-severe disease.	No new evidence identifies a proven curative antiviral; EMA states current treatment remains symptom relief.	Continue supportive care: antipyretics/analgesics, hydration, monitoring, and return precautions.
Hospitalize severe disease.	WHO rapid risk assessment reiterates severe disease can cause organ damage and death.	Admit patients with severe disease, high-risk features, or inability to maintain hydration.
Use laboratory confirmation where available.	WHO 2026 rapid risk assessment clarifies RT-PCR early and IgM later, while noting access and cross-reactivity issues.	Use RT-PCR in week 1 and IgM after week 1; interpret results with epidemiologic and clinical context.
Use crystalloids rather than colloids when IV fluids are needed in severe hospitalized arboviral disease.	No contrary evidence identified.	Continue crystalloid-first approach with frequent reassessment.
Avoid routine corticosteroids in non-severe suspected or confirmed arboviral disease.	No new evidence supports routine corticosteroids for uncomplicated acute chikungunya.	Continue avoiding routine corticosteroids in non-severe acute disease.

Original WHO 2025 recommendation / position	New evidence impact through 29 April 2026	Updated recommendation
Prevention relies on vector control and outbreak preparedness.	PAHO/WHO and WHO emphasize increased transmission risk and need for surveillance and integrated vector control.	Strengthen integrated vector management, community engagement, and surveillance.
Vaccine role still evolving.	VIMKUNYA has favorable short-term immunogenicity and early older-adult safety data; IXCHIQ has important older-adult safety concerns in some settings.	Use vaccines according to national/regulatory recommendations; prefer individualized risk-benefit assessment, especially for older adults, pregnancy, immunocompromise, and travelers.

Sources: [WHO publication page](#), [WHO rapid risk assessment](#), [PAHO/WHO epidemiological alert](#), [EMA IXCHIQ Risk Management Plan](#), [CDC vaccine information for healthcare providers](#), [Eurosurveillance](#).

2.6 Practical Clinical Algorithm as of 29 April 2026

1. **Suspect chikungunya** in a patient with acute fever and severe arthralgia/arthritis, especially with rash, headache, myalgia, travel, residence in an outbreak area, or exposure to *Aedes* mosquitoes.
2. **Assess severity immediately.**
 - Red flags: altered mental status, shock, organ dysfunction, severe dehydration, severe neonatal disease, pregnancy near delivery, significant comorbidities, older age with frailty, or inability to maintain oral intake.
3. **Evaluate differential diagnosis.**
 - Consider dengue, Zika, yellow fever, Oropouche, malaria, measles, rubella, leptospirosis, parvovirus, enterovirus, adenovirus, typhus, streptococcal infection, and rheumatologic disease
[CDC Yellow Book: Chikungunya](#).
4. **Order testing when available.**
 - Illness day 0–7: RT-PCR.
 - After day 7: IgM serology, with awareness of cross-reactivity.
5. **Treat non-severe disease supportively.**
 - Oral hydration.
 - Antipyretic/analgesic therapy.
 - Avoid routine corticosteroids.
 - Provide return precautions for worsening symptoms.
6. **Hospitalize severe disease.**
 - Monitor organ function, hydration, hemodynamics, neurologic status, and complications.
 - Use crystalloid fluids if IV fluids are needed.
7. **Prevent further transmission.**
 - Advise mosquito-bite prevention during acute illness to reduce vector infection.
 - Support household/community vector control.
8. **Consider vaccination only according to current national guidance.**

- VIMKUNYA is currently identified by CDC as the chikungunya vaccine licensed and available in the United States for people aged ≥ 12 years, with individualized use for travelers and laboratory workers [CDC vaccine information for healthcare providers](#).
 - Exercise caution with live-attenuated IXCHIQ in older adults or people with comorbidities where safety concerns or regulatory restrictions apply [CDC Yellow Book: Chikungunya, Valneva](#).
-

2.7 Bottom Line

As of **29 April 2026**, the latest WHO chikungunya-relevant clinical guideline is the **4 July 2025** WHO integrated arboviral disease guideline covering dengue, chikungunya, Zika, and yellow fever. For chikungunya, clinical care remains centered on:

- **Integrated clinical assessment** because arboviral and febrile illnesses overlap.
- **Laboratory confirmation** using RT-PCR in the first week and IgM serology later when available.
- **Supportive treatment** for fever, pain, and hydration.
- **Hospitalization for severe disease** or high-risk presentations.
- **Integrated vector management** and mosquito-bite prevention.
- **Cautious, jurisdiction-specific vaccine use**, with VIMKUNYA showing favorable short-term immunogenicity and early safety data, while IXCHIQ has important post-marketing safety concerns in older adults in some regulatory settings.

WHO has separately promoted meaningful youth engagement in guideline development through an abortion-care guideline case study, but available sources do **not** show that this model was used in the WHO chikungunya/arboviral guideline process [WHO webinar page](#).